

ActInf GuestStream 074.1 ~ Narrative Fallacy and Other Limitations of Psychoanalysis

GuestStream | Narrative Fallacy, Other Limitations of Psychodynamic Case Formulation | 1:35:50

Hello and welcome. This is Active Inference Guest Stream 74.1 on March 6, 2024. We're here discussing Narrative Fallacy in Clinical Psychology and Psychiatry with Alexei Toulczynski and guests. So first there will be a presentation, then we'll have a discussion and look forward to everyone sharing what they have to say on that. So thank you, Alexei, first to you for organizing this and catalyzing it and also for the presentation. So thank you. Go for it.

Well, thank you, Daniel. I'm most grateful to Active Inference Institute and you personally for hosting us and your commentary and advice. I'd like to thank wonderful panel members from many different countries who found the time and expressed interest in being here. And to everyone watching us on YouTube. I'm hopeful to clarify from the get go the terminology because a lot of people I talked to reacted to the word fallacy. But as if I'm trying to kind of talk about truth in psychoanalysis or point out some inaccuracies, this is not my intent at all. Or to the word narrative because it's a vast topic and linguistics are involved, I just wanted to clarify that narrative fallacy, the two words together, is a specific technical term introduced by Nassim Talib in 2007. And so I'll try to keep this discussion sort of on the ground, under the hood. And I'll illustrate this concept, then I will define it. Hopefully we'll discuss it from the standpoint of neuropsychology, Active Inference, and then move on to clinical applications. And I very much look forward to a discussion.

And so I'll proceed. This is the pre print of the paper, possibly some of you had a chance to look at it. And if you don't mind, I'll start with this example. What I'll ask everyone to do is to try to memorize a list of words that I'll give you and observe how you feel while doing so. And we'll do it a couple of times. Is that okay? Sure. Right.

So here we go. So here we go. So Magnolia, 1964, firefighters, club, Chicago, jazz, 10.

What was the last one? 10. Okay, now I will do the same thing. And I will present this to you in a slightly different format. Okay.

On a cold winter day, in 1964, a jazz club Magnolia in Chicago caught on fire. The firefighters were on the scene within 10 minutes.

For most of us, it is a little bit easier to retain the second format. And, you know, we'll talk today about why this is and what's going on here.

And then I'm just using this as an illustration. The example is from the repeated battery for neuropsychological assessment, R-BANs. I modified it slightly, but this is sort of what we'll be talking about. Usually in these conversations, Miller and this is the case of the repeated battery for neuropsychological assessment.

We'll talk about this. We'll talk about this.

We'll talk about this.

Usually in these conversations, Miller comes up and Mark Solms calls this Miller's law. I mean, I think it's a

Daniel Kahneman But you know, he makes a point stating in some textbooks that a relationship between working memory, long-term memory, action, perception, semantics is complex, flexible and interactive. So these systems are intertwined. If I may, you know, use a computer science metaphor, then long-term memory in a computer is hard drive. It is a spacious and cheaper memory system. Working memory is more flexible, faster, and more expensive. And in computers, it's called RAM, random access memory. So moving forward. The long-term memory, the hard drive, if you wish, this is a very old model that I think also Mark Solms uses quite a bit in his theories. When he says repress never returns, he relies on this chart quite a bit. It was introduced by Squire in 1992, who stated that, you know, long-term memory can be divided into explicit or declarative memory and

implicit non-declarative memory. And if we open the working memory, you know, component, then this model has been revised and there's some alternative versions. But one of the current views is that it consists of these subsystems. One of the simple ones is the phonological loop, where we simply take in auditory information, such as musical sounds or speech. It's not a huge space. It's about two or three minutes of information. And the visual-spatial sketchpad, where we process visual and spatial information.

And the central executive is the coordinator and, you know, it allocates attention. This is the most resource-intensive component of the working memory. But the newer component that would be introduced later is called the episodic buffer. And this is not a monosensory space. This is a multi-sensory space. One of the sort of mysteries was that the phonological loop is two or three units, and we can retain seven. So this is one of the reasons why the episodic buffer is introduced. But it can be used to bind things together between senses, such as, you know, visual and auditory data.

And importantly, if you look at that chart, you can observe that this binding can occur without the deployment of the expensive central executive system. You can see that there are connections from visual to the phonological loop directly without the deployment of expensive central executive resources.

I've already used the term binding, and it simply refers to linking the features into objects, such as color red and shape square. We quickly create an object red square, and then we stop thinking about it. In terms of processing speech, you know, the two terms that are pertinent to us are within sentence binding and across sentence binding. And as the name suggests, within sentence binding refers to using our storage language knowledge to apply to temporary retention of single sentences in a relatively automatic manner without using expensive, you know, attentional resources.

And across sentence binding is the same thing where we connect together multiple sentences.

I will illustrate both again on the example that I already gave you.

I will see you on a cold winter day in 1964, a jazz club magnolia in Chicago caught on fire.

The firefighters were on the scene within 10 minutes. If we slow down here and see what's happening when we hear this, if I start reading on a cold winter, then if you're familiar with American English, you know that the next word coming is day. And that's a prediction that is based on your knowledge of language, a semantic prediction, if you wish. Similarly towards the end, you can hear firefighters were on the scene within 10, you know that the next word coming is minutes. I'm already introducing the concept of redundancy in speech, sequential redundancy, if you wish. And here I was an example of a semantic one. And tactically, because you know the structure of affirmative sentence in English, after you hear the subject, a jazz club magnolia, you know that the verb is coming and you're waiting for it. You will also chunk on a cold winter day together, you will chunk jazz club magnolia together and within 10 minutes. And of course, you'll chunk prepositions and articles together with nouns.

And 1964 semantically is just one unit of information, not three different ones. Phonetically, in English, it's four syllables, in Russian it's 12. So we save resources quite a bit. If you wish, we're doing compression, you know. Across sentence binding is working, but only when the story is coherent. And you can see here in this example that the two sentences are causally related. There is also a temporal structure where there was the fire first and firefighter second. And there's a meaning, you know,

when we read the story or hear it, we're wondering about whether the firefighters were in the scene quickly enough. And then you hear a confirmation of this prediction that they were on the scene quickly. All of that helps us process the information. Now we can fiddle with that a little bit and introduce a meaningless but syntactically correct sentence, such as the Chomsky example, colorless green ideas sleep furiously. Not very easy to retain because we're trying to make sense of it, and it's not working and we stay with it a while. Or here's a meaningful but syntactically problematic sentence. This is a poem from Anne Stevenson called Utah. Beside him, the gray-headed man has let one arm slide awkwardly over his shoulder. He's talking and pointing at whatever it is. So it's not just the length of this phrase, but the way it's structured makes it problematic for us to retain it. So you need both coherence and meaning. If, however, you're dealing with the very first example of 10 words that are unrelated that I gave you, I'm not saying it's impossible to bind them. It is possible, but it will be more resource demanding, more expensive, so to speak. These are formal definitions of within sentence binding and across sentence binding. Again, the important part is we do this relatively automatically without additional costs of expensive, if you wish, are metabolically costly resources under certain assumptions. And similarly, we can do a cross-sentence binding when we have a coherent and meaningful story. Here is the kicker that, according to Daniel Kahneman, and we can look at many other perspectives, including Mark Solms in his book Hidden Spring, brain-mind is an effective or efficient, or you could call it lazy processing unit, which means that we tend to fall back to simplest possible processing regime. And we only deploy complex processing when we have to. In terms of emotions, there is an effect that accompanies effort, which is strain. It doesn't feel so good when we apply effort. And if you have a video camera across your face with a relatively fast frame rate, if you're trying to do 17 times 69 in your mind, you will see that your pupils dilute. Unless you have iconic memory, unless you're a savant, then for most people it takes some effort. And as a speculation, why is it that we have unpleasant effect when we apply effort? You know, possibly this is so because it is dangerous to operate on nearly depleted resources for more vulnerable and less resilient. And Kahneman's example is that if you are doing 17 times 69 in your head, while trying to make a left turn into traffic, you're more likely to run into an accident. So as a result, if our activity demands that we do complex processing, then it means conscious commitment to doing complex processing. In neuropsychological terms, that probably means impulse inhibition. We need to inhibit the impulse to fall back to a simpler regime. And then we're probably, well, because we're human to this universal phenomenon of fatigue, very single functional task in the brain mind that we do, we'll get tired, such as impulse inhibition. This is why when people walk out of the gym, they sometimes eat a donut. Now it pertains to therapy very strongly. You know, we're tired on Friday night, you know, compassion fatigue, auditory fatigue, you know, we crave silence. So what I'm trying to say is no matter how trained the therapist is, no matter, you know, how experienced and skilled, none of us are superhuman beings with unlimited wicked memory, and we're all prone to narrative

fallacy at certain times. The two obstacles that I see on the way of this discussion, one is simpler, which is people being unaware of the issue. And this is easier to correct when we just talk about it. But I've also faced some denial where people flat out say that this is a non-issue and what are you talking about? And there is no problem here. And this is why I think that the primary effort of the paper was to kind of bring awareness to this and to discuss it. I could be wrong, but I'd like to hear substantive arguments about this. Another interesting, you know, connection point before we move on to clinical is the thought by Mark Solms, which he voiced years ago, I don't recall even in which seminar. It's about the fact that how we think influences how we work in a clinical practice. And this is a very broad term, this phrase is sounds so simple that it may be even appearing trivial, but it's not. Well, for example, what we know and what we don't know influences it. If you can imagine a clinician who happens to be unaware that thyroid dysfunction may present as an anxiety disorder or depression, then they will never ask this question to their patient. When was the last time they served primary care doctor and did thyroid panel blood work? You know, or some clinicians are taught to not ask questions because they're experiential and they derive conclusions based on experience and then they don't know. It could be treating Hashimoto disease and therapy, you know, until cows come home. You know, personal therapy changes how we think, whether we can think flexibly, such as using categories with discrete data, dead or alive, and using continuous thinking with continuous data, such as temperature. Can we even think statistically? Do we know how to do that? Can we think in terms of dynamics, complexity and emergence, or we think mostly in deterministic and static ways, all of these things matter and they influence how we work. So a related concept is a case formulation. And I wanted to mention that this is not unique to psychotherapy that it's used in medicine. Anecdotally, you know, experienced neurologists can start suspecting Parkinson's before the patient walks into the consulting room because he, he is shuffling outside the office, right? And then the patient walks in and information is collected and you see rigid, you know, maybe flat affect, rigid posture, difficulty initiating movement, cognitive slowness, and shuffling gait and resting tremor. And then the neurologist is inclined to say that I think that you might have Parkinson's disorder, Parkinson's disease, which is a formulation. This is a sentence. This is a story. And the gap explanatory gap was filled in from what the neurologist knows from his training. And formally, a primary tool, you know, this is the definition. A case formulation is a hypothesis about the causes, precipitants, and maintaining influences of persons' psychological, interpersonal, and behavioral problems. And, you know, I think personally that it's one of the primary tools in conceptualizing what's going on with the patient. And it's very complex. The little vignettes that you'll hear in a few minutes are oversimplified, shortened, and a little artificial. You know, it, it usually takes several sessions to come up with a preliminary formulation. But to be very honest, among many other things, a case formulation is a story among its other functions. So here's a short case. This is taken from a paper by Perry and colleagues in 2006 called Psychodynamic Formulation, Its Purpose, Structure, and Clinical Application. Mr. A is a 52-year-old married businessman. He presents on his own initiative with a depressive syndrome after being once again passed over for promotion. He himself does not

understand this rejection, but it is probably related to his lifelong tendencies to procrastinate and to annoy his superiors either by being obsequious or by challenging their authority. He has a history of two untreated depressive syndromes, one in his 30s that also followed a professional failure, and one in his 40s that followed his son's defiant marriage to a woman of another religion. Mr. A's father was a sickly, professionally frustrated, type A personality who died of a heart attack when Mr. A was in his scenes. His mother has always been a martyr with smoldering despair characterized by chronic insomnia, self-doubt, obsessive ruminations, and social withdrawal. She never sought treatment. Mr. A has essential hypertension, for which he takes a field top of 250 milligrams. His mother's history suggests a genetic predisposition for a unipolar depression. Let's imagine that this is all the information that we have. Now, let's take this to a psychologist, a psychodynamic clinician who was trained in the tradition of ego psychology. It goes back to Sigmund Freud, was elaborated by Anna Freud, Charles Brenner, and others. And this is a formulation. I want to point out that in each of those examples, you will hear a highlight. And Mark Solms does this. You'll hear the central theme, the main point. So in ego psychology, you may

hear something like this. Mr. A's central conflict is between an unconscious wish to kill off his competitors and an unconscious fear that he will be killed if he acts on that wish. Whenever he expresses derivations of his competitive wish directly, he becomes frightened of retaliation. He therefore resorts to expressing the wish indirectly by passive aggressive maneuvers, such as procrastination. Conversely, whenever he responds to this fear of retaliation by being solicitous and obedient, he inwardly feels resentful and diminished. To contain the struggle, Mr. A has developed intellectual mechanisms that, although adaptive for certain aspects of his work, are maladaptive interpersonally, and then they isolate him emotionally from others. I apologize to, you know, you know, esteemed psychoanalysts on the panel. But, you know, for the audience, I just want to highlight that in ego psychology, you will hear conflict, you know, a formulation will contain conflict, typically between a wish and a fear. But that's sort of the style. Let's look at the same exact patient from the self-psychological standpoint that goes back to Heinz' cohort and was elaborated by Gero and others.

Mr. A's central problem consists of his low self-esteem and consequent need for continual recognition and approval from others, along with his inability to accept any limitations either in himself, which led to disapproval from others, or in others, which reduce the value of their approval when it occurs. Presumably during childhood, his depressive mother and sickly father were so self-absorbed with their problems that they were unable to respond empathically to his age-appropriate aspirations. At the same time, both parents narcissistically invested in their son in the hopes that his achievement would make up for their failures. In self-psychology, you typically hear about narcissistic vulnerability, difficulty idealizing parent, difficulty in mirroring, empathic attunement, and perhaps narcissistic extension. Here's a third version. This is object relations school, going back to Melanie Klein.

Mr. L's central problem is the failure to integrate the good and the bad representations of himself and others. During childhood, his depressed mother could not respond to her son's needs and demands. Mr. A is

unconsciously frightened that his resultant rage would destroy the very one on whom he depended. He repressed

his bad, angry self and acted like a good, obedient son. The splitting was reinforced by interactions with a controlling father, who unitively viewed any of Mr. A's independent assertions as acts of defiance. In object relations, you will hear typically about splitting or lack of integration.

I want to say that you hear some terminology and I don't want to caricature it. Each of those formulations is complex, nuanced, and indeed useful. You could get to good results in each of those directions of the work. But they are very different. And all I'm trying to say is you can observe that they are influenced by how people were taught and what they studied. I want to ask you just to reflect how many of these formulations seem to be elaborated and detailed or brief and tentative. How certain do you

think the author of each narrative is? Even if they use euphemisms like it seems that it appears that, do they sound uncertain in their hypothesis? This is a CBT example from an authoritative source. CBT stands for

cognitive behavioral therapy. This is American Psychological Association Essential Components of CBT for Depression. So nearly textbook. Different patient but also depression. I suggested to Nancy that I spend a few minutes in the session giving her a first notion about how CBT would address her difficulties to help her decide whether she wanted to pursue it. She agreed. To teach the cognitive model, I mapped onto a thought record, the situation Nancy described to me earlier when her roommate Connie looked hurt when Nancy turned down Connie's invitation to go out to dinner. I asked Nancy a short series of Socratic questions to show her how these thoughts made her feel guilty, inadequate, and trapped.

So in a CBT example, this is more cognitive therapy than behavioral, you see that the thoughts are being analyzed and are put into position of kind of causes that the patterns of her thoughts are causing this symptoms. You had conflict in ego psych, you had narcissistic vulnerability and lack of idealization of the self-psych, you had splitting, here you have thoughts. Here's the final perspective, quite oversimplified and artificial, but at the same time while you and I, you know, we all talk in here, there's many psychiatrists in the world who diagnose their patients with major depressive disorder and prescribe SSRIs. Here's the story of chemical imbalance. This is chemical imbalance in their minds leading to the symptoms. This is based on a story called a monoamine theory of depression.

And these, you know, clinicians were thoughtful and, you know, trained in well-meaning. They've read many peer-reviewed studies with some statistical results of SSRIs. We now know that the effect is weak, that it's 0.3 effect size compared to 0.9 of dynamic therapy for depression, but nonetheless, there's modest effect. The important part is no psychiatrist will be able to explain to you the causal chain of events from a molecular level of serotonin all the way to the macro phenomenon of depression that is transient, you know, emergent and, you know, complex and dynamic. So then the connection from molecular level of serotonin to the symptoms was made exclusively through the clinician's theory and nothing else. This is an example of also narrative fallacy.

I want to also say a few words, there's more of that in the paper about verifiability.

So once the preliminary hypotheses are formed, the question is, can we truly test them? And if you remember the texts from each school of psychoanalysis, then I say that they're not specific enough and not falsifiable enough. If you take the, you know, scientific statement that is falsifiable, such as water boils under normal pressure at 100 degrees centigrade, it would take a single observation to disprove it. You can observe water under normal pressure boiling at 90 degrees and you're done.

Dr. David Pérez Vandenbergheer But none of the statements you heard before are falsifiable in that way. And also many were really not specific. Now, even if the psychiatrist, let's say, uses response to medication as data, you know, if, if you're saying something like I gave somebody Ritalin and they're better, therefore they have ADHD, this is flawed because if you give a stimulant to anybody, they will attend better. And therefore it doesn't prove that a person has ADHD. This is an example of circular logic. Another obstacle on our path is Barnum effect and Fourier effect. It's the same thing, but I think that people don't talk about it enough. Essentially what it means is if you feel resonance with a certain idea or paragraph or a patient feels resonance, that says absolutely nothing about how clinically useful it is, accurate and, and none of that. Here's an illustration of what actually Voror did. He gave his students personality questionnaire and inventory. They answered it honestly. And then he mailed them results which were identical to one another. And he took these results from an astrology book. Subsequently, he asked them to rate how they compared these texts to how they felt about themselves, how accurately, you know, they thought it was on a one to five scale. And they rated it as four point something, I think 4.5. Now, astrologists milk that effect, like there's no tomorrow. This is their livelihood. There's a way to present a text with certain things in it, like detail. It has to be detailed. And then when you read it, something will resonate. But it absolutely doesn't mean that it's accurate. The third bullet is effort justification bias. And I think that the hypothesis you heard before, you know, they took a bit of work to formulate and a bit of thinking, which is effort. And it is not that easy to let go of something that you developed with effort. And finally, some clinicians use what's called response to intervention to decide to gauge if they're in the right track. For example, if you deliver an interpretation and the patient cries, you tend to think that you're accurate. And I think it's a very error prone method so that if somebody punches me in the face and I cry, it doesn't mean that the intervention was therapeutic, it could be iatrogenic. And there's many more details on that in the paper. But to summarize, I think that sadly we do not have a rigorous independently verifiable framework of hypothesis testing. We think that we do. We declare that we're hypothetically deductive. But I think if we go down to it, it is not.

Moving on to active inference perspective, and I apologize to, you know, analysts who are, you know, not close to this framework or anyone in the audience, but the idea that I shared before that brain mind tends to resort to simpler regimes of processing and only deploys complexity when absolutely necessary is not news to active inference community. This is a formula for variational free energy. And you see that it can be decomposed as complexity minus accuracy. So if you minimize variational free energy, then complexity is minimized. And this is the principle of Ockham's Razor, where you are seeking an accurate explanation. But among several, you will pick the one that is simplest.

There is certainly the word inference and belief is completely kosher and good in active inference community. And this is a big part of the model. However, all inferences are probabilistic, they're statistical, and they're quantitatively verified. And I don't think none of the hypothesis I showed before are probabilistic and quantitatively verified. There's a viewpoint on emotional inferences in active inference community. And from that perspective, you could say that anxiety is just a commitment to a Bayesian belief that I am anxious, which best explains the prevailing sensory and interoceptive cues. Finally, you can look at the some of the processes happening in psychotherapy from the active inference standpoint. Perhaps before the patient and therapist meet, therapist has certain generative models.

We learn how shame is different from guilt, how acute fear is different from anxiety, that there's different kinds of anxiety. And then we meet a patient, and we have to adjust our models based on the specific information

coming at us, which is, again, perception and adjustment of the models. At some point, we could infer if we're feeling

something in therapy, say counter transference, what is it that I'm feeling? That's an inference. Or what is it that the patient is feeling? And then we could act even silently, if we label it in our minds that I'm feeling fear right now without saying it out loud. That's an action, mental action. And so this framework of model infer act fits very well in the active inference framework. The take home message, I think, if we look at narrative fallacy, is that when we're driven by theory more so than by the patient's data, then we tend to perceive less and act more. We're modifying the environment based on overly confident beliefs. And such as the example of a CBT clinician teaching the patient, you know, they're teaching the patient, right? Another issue is that a very high level of precision, high certainty in internal beliefs.

And that violates the principle of maximum entropy by James, which is cited by Carl Fristen. What this states is that internal beliefs have broad entropy, high entropy. The sensory entropy is minimized, but the entropy of beliefs is maximized. The next issue is that, okay, when we have data coming to us that it either confirms our predictions or disconfirms them, we know what to do. But if there's no data coming out, when there's, you know, uncertainty, it is important to not act, which corresponds to Freud's principle of abstinence, until we have enough data to say something or to correct the beliefs.

So, and just to summarize, in my opinion, and I'll be happy to be corrected, the formulations above and in the peer-reviewed paper are not high entropy. They're confident, they're elaborated, they contain conclusions based upon conclusions, while the whole thing was based on assumptions. So, we can declare and state that we're not attached to the theory, but I don't know if that is so in clinical practice.

What else do we have after all to fill in the gap? So, I think we can try to conclude that it is important to stay data-driven more so than theory-driven, that we need to be trying to test our inferences in the environment to the degree possible. And for that, they need to be formulated in a falsifiable way. It's important to remember that inference is a probabilistic, which requires and necessitates the ability to think statistically and not only in the story form. Also, I want to say that what happens when there is, you know, one school of thought or identification, I quite often hear from

their experienced clinicians, I am a Kleinian analyst, or I am an EMDR therapist, or I use effective neuroscience to infer what is the main emotional problem for the patient. So, in these circumstances, I think the chance of narrative fallacy is higher because we are operating within this one school. Now, if we're able to take multiple viewpoints and stay meta to all of them, we can see that that would decrease the probability because sometimes we can use, you know, Ekman perspective and Tomkins and talk about shame being central in narcissism. This is not what you're going to hear if you're studying neuropsychanalysis and clinical workshop. You will hear about the play system, right? We don't talk about shame enough. So, I think awareness about narrative fallacy and Barnum effect allows us to resist the temptation of confusing coherent and meaningful as clinically accurate and useful. Coherent and meaningful texts in psychoanalytic language are seductive. So, I hope to have a sort of discussion about narrative fallacy. I think that if we connect the dots by using the glue of the theory du jour and nothing else, it's a problem. I think that we can consider updating how we do case formulations and move away from exclusively story-driven form. You can already find it in Macaulaym's latest book, Psychoanalytic Supervision. She uses 10-dimensional assessment. Some variables are continuous, some are discrete, but, you know, I think that, you know, it moves away from the story form in some domains of learning. And I think also talking honestly about possibility of using tools in our work in addition to narratives is important. Like neurologists talk to the patient and they examine the patient, but they have MRI, PET scan, EEG. And these are completely different ways of analysis than talking to a patient. And I certainly think that we can teach these things differently. I wanted to share that so far I haven't heard any pushback from CBT or psychiatrists, CBT colleagues or psychiatrists, but some of my psychoanalytic colleagues and psychodynamic colleagues expressed pushback and I classified it in four different domains. The first one is the most frequent thing you hear from psychoanalysis. You painted a straw man. The second one is that I, Alexei, I'm not a psychoanalyst and I have not read enough Freud, Lacan, Klein, Kohut, etc. Some people say that psychoanalysis is art and not science. Also quite frequently I hear, I don't work this way, which in some cases took a form of I don't formulate cases. And I struggle with that actually because if we do clinical work and our patients have some probability of a complete suicide, I wonder if this clinician honestly doesn't think, if they only intuit and feel things, right? Some people say this is an American thing, what you're talking about. We don't do it in our country. We work with the transference, psychoanalytic field theory, Chimeris. And I flat out heard, you know, there is no fallacy. So I think that these are defensive reactions. I'm not sure why, because I'm saying it applies generally, not only to dynamic work, but to CBT and psychiatry, but also to journalism, as as I pointed out to economics. And I'm hopeful to maybe have a discussion about it. And we are right here so we can discuss. Thank you.

Great. Thank you. All right. Let us go to any of our panel in order who can say hello and give any first remarks and we'll just kind of go around and from there.

If there's any chance to start with Andrea, I would be most grateful. He wrote a commentary.

Great. I'll read the comment in the chat. Andrea wrote, Alexi, very interesting and sorry I have to leave, but I'm sure you will discuss on the major role of affects in the memorization and in the binding of the different components in the anxiety provoking story of the jazz club fire as compared to the list to memorize, whereas the affect of coloring is completely absent.

Could you say more about that?

Yes, I think Andrea is bringing up. Is sound working okay?

Yeah.

Is bringing up an important part. I think Christina Alberini and others talk about it. When there is effective tone in the messages, we tend to retain information stronger and better.

So that if fire evokes fear or anxiety in somebody, it will be a stronger memory.

And certainly what I've mentioned are very simplistic ways of looking at things.

But I think when emotions are present, when we memorize things, here's a Mark Solmes example.

You know, when children do two fingers in power outlet, single exposure, lifetime memory, intense effect, pain, you know, doesn't take any repetition, single exposure memory for life, phobic memory.

So I think that perhaps I overlooked this important part, but yeah, I think it is important to mention and to add to this discussion.

Yeah.

Go ahead and unmute David and then go for it.

And then go for it.

So on the affect aspect of narrative, seems to me that's that I listen to the narrative and then go for it.

I listen to whatever I hear from the perspective of an integrative, unitary self.

And that self is conscious and unconscious, but it's an act of my brain.

it and it seems to me that narratives have to include all the components of consciousness in them and that so i would like to talk more about the issue of the why you know you talk to us

Alexi in a narrative way and we respond in that way there's no way to eliminate it

i i think the easy part of your discussion it's the second point is dealing with case formulations

um uh which i think you would agree with but my position is the case formulations are

good for communication with yourself or with others uh but are not that are they're contradictory to

listening and proper the ideal level of involvement and you know it's great to learn all those stories

and have them in the back of your head but not to be thinking about them uh because they get in the way

of your responsiveness and your ability to listen properly and i feel like you know i've been at

this now for 45 years and well i it's like a comfortable chair that i just sit down and um

attune myself to whatever it is that is my way of analytic listening and and

and participating and i i might think about a particular patient maybe if it strikes me as

self-psychology descriptor uh or something like that but i think the case formulations

get in the way of actively doing psychotherapy

um

would you like me to respond there sure please uh you've said many useful things david and uh i think that um um

uh we may not formulate and listen to the patient at exactly the same milliseconds uh but you know i think usually i'm quoting mark's homes again when people show up in dynamic therapy or psychoanalytic therapy this is uh a referral request that contains an explanatory gap when the patient walks in and says i'm afraid of elevators i don't want to ride an elevator don't even worry about my mom and dad and whatever this is a very concrete form and they don't need dynamic therapy they can do exposure

when they have ocd and they want to walk out the house you know they need exposure when people come in and they say

i feel badly but i don't know what it is and how it is or i'm doing something i know i shouldn't be doing it but i'm doing it anyway

so and part of the work of course we'll listen for a long time for as long as it takes we'll think together with a patient and come up with some hypothesis about what this is

and uh i think it's inevitable i mean if we bring clinical work so 25 of mental health professionals experience a completed suicide of a patient in our careers

if we're dealing with somebody who has acute ptsd this is not like speculation but they've been tortured and they have flashbacks and nightmares and the whole nine years

probability of a completed suicide is five times higher so i must think i have to think at least to do risk analysis do they need hospitalization right now or other things

and i think that our minds are never fully blank when we're listening i think that's an illusion from the honest like you know no memory no agenda

it's good to declare that but our minds are not blank when we listen and we think while listening so this is all what intertwine so

so so i agree my mind is not blank is i'm just not aware of how it's tuned to what i'm doing it's like an instrument and

i agree with you that at various moments in the work with patients we need to have explanations to things and then a formulation can be abused in some way uh i'm not talking about in psychoanalytic language but what is it at the level of description or communication when those moments occur in therapy

what is it at the level of description or in therapy then some formulation is needed and but i'm talking about the psychoanalytic schools of thought as describing a patient in a formulation is good to have the knowledge for all the different schools and to find you find your own way of listening to the patient

the patient in a way of listening to the patient in a way of listening to the patient in a way of listening you know at some point i mean if we were just listening then patient can talk to a wall right

you know some point we do things such as clarification confrontation interpretation

you know containment and these things all of them should be driven by working understanding of what's going on if we've got no formulation in our heads we have no compass on how to act in therapy even if you listen for i didn't say it's not in my head sure i'm not conscious of it right right right yeah yeah but

i think i've said enough and i think david and i will mostly agree on many different things yeah but uh thank you maybe yeah um anyone else has any comments or questions critique anything um shall i uh chime in there

okay um well first off thank you for having me um

um i prepared uh some comments um i'm afraid alexa you will classify me in the defensive people i i'll try to seduce you not to do so but we'll see um so um alexei of course you are more than right when you warn for the problem uh page 11 of hyper salience where we draw connections between the pieces of data without any reason other than our theory driven beliefs or i would say more generally when we understand uh between quotes uh the problem of our patient you know i'm a lakarian so uh jacques famously warned in the same vein the clinicians or maybe everybody beware of understanding that's what he said beware of understanding so i do profoundly not think that clinical work is about understanding at least not at his conscious cognitive semantic level and i also agree that the monoamine theory of depression has the same logical status from that standpoint as the discussion of a state of panic from the standpoint of audible dynamics on page 24. and i will try to show how one can do clinical work without making interpretations not sure if that's gonna hold um so not to take too much time i had prepared um a paragraph about the importance of transference so i'm just gonna name that uh paragraph um i'm sure you're aware of this importance but you just said something to david that everything that we do as clinicians in our third in our therapeutic session i should be driven by hypothesis about the patient and i do think that whoever is the patient um there are some like universal principles uh to work with patients uh to work with patients and one of them is um to be unconditional uh and so many of the actions taken in therapy from the clinician is actually driven by this principle of being unconditional and other principles in the same vein which are some sometimes called the solomon principles presence proximity patience availability these kind of things so i i really do think that establishing a transference relationship and i i wouldn't think about transference and counter-transference a transference we're both in there although the relationship is not symmetrical um is um establishing uh an apparatus which will work or might work uh for therapy and that many of the actions of the the therapists um are aimed at uh establishing this apparatus so my third paragraph is of course that there is more to the clinical work than the transference relationship uh you could say the transference relationship is how the therapist engages in a therapy with his subjective apparatus but there is also the intellectual investment of the clinician and to me that is not about interpretation at least not interpretation as is given on page 12 of the person in the self about the ego weaknesses etc in dutch that is the language in which i first did clinical work there are actually two words there is interpretation and dirty which resonates with the german the truth like in the trump trump trump interpretation interpretation is an addition of content while deuting is a form of pointing underscoring punctuation so i have prepared a slide uh to show what i mean um i'm not sure

i can i will try to um um um yeah

uh thank you um

yeah let's see what that gives

um let me see

i can do like that is that fine too yep that's fine thank you i'll drop it so it's visible

so three little cases um in in which

deutung and maybe it's eu instead of eu i have a doubt now has given effects um

maybe these are effects that you would qualify as policy but let me explain so early on in my practice i

had a patient who had uh troubles uh with uh anxious obsessions whenever she got anxious

she started to count one two three etc and she also would have specific rituals which involved the bed

um and especially which involved um drinking cups and she would with her finger go around the drinking cup from left to right and from right to left and count and go around the handle from left from down to uh from up to

down from down to up and count and it just the order that struck me because in dutch the word for

a drinking cup is the same word as for head and the word for the handle is the same word as for ear

so my patient was talking about in dutch bet copy or tube and i only said that bet copy or she let me say that she

told her story and i picked up these words that these are words that she said and i only picked up

these words and i put them in a sequence that copy or tube because it struck me and this was enough

this was enough it was early on in my practice and quite spectacular to open up like you would have cut somewhere where suddenly everything comes out of it where that started a chain of associations

in which she then recounted how as a little kid she had anesthesia and anesthesia was done at the level of her head and at the level of her ear and that she was extremely anxious that she would not wake up

and that she had learned to soothe herself by counting so that she would count until she went to sleep and

that she would as soon as she would as soon as she would take up the count again as a kind of way to make a chain a thread that kept her alive so this was spectacular because the whole session was one

association after the other about these hospital experiences and about other connections she was making

and it the the the the session was not enough to contain her uh she came home and she wrote an email i got an

email with uh other associations and uh and at the new session she started over again so yeah at least you could say

this made the patient move the patient moved she she moved mentally um so that was something let's say it was

something in another case um my patient another patient um spoke about her life and her difficulties and also about her childhood and her upbringing and she was brought up in a religious community and they had

rituals and prayers and one of these prayers said uh it was one of the prayers he had she had kept in

memory uh jesus in the middle so this in dutch is said jesus in the middle so what i only said is i only

fragmented it differently i said jesus in admittance and that changed the meaning from jesus in admittance

from jesus in the middle to your sister in the middle so again this sparked associations and uh

associations about her competitive feelings towards her sister and a mental movement and the third example is an example not of my practice but it's been published by a psychoanalyst and the kenyan psychoanalyst his name is patrick gautier lafaye and he speaks about a middle-aged patient who who who comes to his practice and uh says to him i'm almost ashamed to come and ask for your time and attention because everything goes quite well in my life i have a happy marriage i have a happy happy job i have good children but it so happens that now and then seemingly from nowhere i have these heavy moods and these depressions and i don't know how it comes so she counts her story like all the patients do and at one point she says in some kind of context where she speaks about her mother my my mother couldn't in in in french uh ma mer n'est pas parvenu à but instead of saying ma mer n'est pas parvenu à

she says she has a little pose on an awkward moment in the sentence she says ma mer n'est pas parvenu à so normally you would say in french ma mer n'est pas parvenu à and she says ma mer n'est pas parvenu à and the analyst who is a lecanian hears this and it is awkward this is not why he would the way he would speak this sentence and he said the only thing he does is he repeats that ma mer n'est pas parvenu à and then she hears what she said because this changes the sentence the papa revenu

it misses it's missing in my slide it's the the key means uh daddy came back

and so when she hears what she said daddy came back she suddenly starts to cry she had already counted the story of her father leaving her and her mother when she was young and her mother being alone to raise her um and he and she being very loyal uh um with her to her mother but now that she hears father comes back she realizes that she actually had longed for the for her father coming back but that is longing she couldn't let go of it or she couldn't express it because that would be like not being loyal to her mother who was alone and who was um angry towards her husband having left her and she felt the duty to be on the mother's side but being at her mother's side she she couldn't um process her longing of her father coming back and uh her sadness about his absence she couldn't process that because there was no place for that and now in the analysis she she could do that

uh i see your hands is raising and i know i'm being talking already a lot but these are three examples in which mental movement is induced without adding semantic uh material it's it's about in it's about doing about pointing to something so i had two other paragraphs three other paragraphs but one small uh so but i first uh leave you uh you you raised your hand so i guess you want to say something alexis if that if that's okay with everybody yeah so arian i i think that i want to separate two things one is i honestly admire your work i say it absolutely sincerely without flattery that these are creative interventions and i love hearing vignettes like this i've had similar moments where i went to a visual metaphor it opened up the work i think you're a fantastic uh clinician and analyst and your patients are very fortunate to be in your care uh i that's that's one statement that i honestly mean the second statement is i don't know if we can use these vignettes to uh generalize i don't know actually well first of all when you says the patient had anxious obsessions and rituals you are already formulating these are beginnings of a formulation you classified her behavior as a ritual not as a tick you know and so you've thought about it okay but then when you said something about these you know

phrases in a certain order or not and the patient reacted by opening up and you know recalling the memory in the hospital what we don't know arienne is whether it was your words or your non-verbals or the six months of analysis that happened before then and the containment that you created in the relationship i don't really have grounds for a causal inference that it's exactly that interpretation that caused the effect and finally i think that if we look at case studies this is absolutely powerful material but certainly not grounds for uh causal statements because again all clinical reports are correlational data and i think i've used that example with you from research book by kastin that if i you know um claim that watching television for a week cures common cold i could have 30 years of experiments i could have tens of thousands of patients each one of them with a proper common cold you know shows up watches television for a week comes out there's no common cold therefore television cures

common cold we don't have data for causal statements we don't know what exactly helped and specifically whether interpretations are not interpretations we have a school in an in in the us called new school of psychoanalysis with spotness and they flat out say interpretations don't work we don't interpret they also say that their field of psychoanalysis works for schizophrenia but there is zero research about their specific school of psychoanalysis versus object relations versus lacanian versus isdp nobody has done that study we have general studies on dynamic treatment but what style of dynamic treatment in a randomized clinical trial was never done so the the i think the scientific answer is we don't know exactly what worked and finally i think if we're talking about improvement of a patient with you know anxious obsessions and rituals another thing the the the good thing is uh um follow-up so if we wait six months after the work is done and we collect thousand patients and we know that the results stay for these thousand patients then we have something to say about the efficacy of this particular modality of treatment if we don't have follow-up in the session they go up they go down they progress they regress so i don't know if we can have grounds to talk about you know clinical improvements that's sort of my take and again to go back in terms of how you work you presented very sophisticated vignettes and this is this is beautiful among other things but i just don't know if it applies directly right so i don't i'm not sure of the format of this because of course i could reply on each point and i would like to reply but um maybe the moderator can come in here um it can be however you see it definitely plants seeds to be explored but how about albert first give a related or a totally different perspective and let you see where we're at in a few minutes well i would say first of all thank you very much alex for the excellent paper um and for the present presentation i'm really loving the discussion so far as well um i had some questions prepared as well but most of them have already been addressed uh uh by all by you and the panel so it's kind of like i'm thinking of something significant i can say i think i'm really curious about the discussion between are young and you right now so but one thing i was curious about is in the what we can do section of your paper um and this is something you also mentioned uh during your um during your speech is uh well besides the audio verbal exchange between the patient and therapist we can use like additional tools and i was i was um curious about like the objective status of those tools and the data you

you acquire from these tools because i'm guessing you also have to integrate those into like a narrative so what does that mean about uh yeah what does that say that's one thing and secondly like how do you um how can you present that to the patient and that's something you you you comment on in the third point of the last part it's it's it's about like some kinds of types of therapy like Beatrice Beebe they film their patients and the baby and ISTDP does this thing as well but it it's i'm curious about have you thought about like how would you integrate these kinds of data but by all means i'm very curious on on your comments as well so yeah

perhaps i can respond briefly and uh you know this is a controversial topic that evokes much debate but if we go back to neurologist right let's say the neurologist here is a patient case where the patient is unable to kind of cross the street properly and they have profound spatial deficits and they they are sort of struggling from that problem they cannot do mathematics okay so they don't know exactly what's going on they examine the patient and ask them to walk back and forth they talk to the patient they may even order a neuropsychological assessment but then they order an imaging test such as an mri and they detect that there's a lemon size you know lesion in the right hemisphere this is not a conversation this is a different uh data point and then they know the etiology they know what's up that the spatial deficit was related to this lesion in the right hemisphere we don't do anything like that in dynamic treatment not at all we insist that speaking to the patient listening to the patient is it it's enough okay in fact if you are talking about video cameras you know in in some modalities of dynamic treatment such as isdp that's a standard practice they insist and kind of it's you know you sign the releases and everything but video cameras are used for supervision and learning i think in psychoanalysis proper that is considered kind of anathema because you know first of all it's intrusive there's a third in the in the room which is a camera and that it sort of privacy concerns and then you know it distorts the transference dynamics and everything but you know what then you're left with is words and also non-verbals like the tone of voice or the posture of the patient but we're missing a lot by processing the data coming at us through the work in memory that is quite limited so uh just as an example again beatrice bb beatrice bb works with uh infant mother dyad where there is no language at all you know there is non-verbal exchange a dance baby's doing google and mom is doing gaga and sort of uh because it is so fast then they do a video and then they slow play the video with mom together and they see the level of attunement the speed if you wish if the baby did this and mom didn't react or the baby didn't do anything and the mom reacted and you can talk about attunement or miss attunement in that play between the baby and the mom beatrice bb is a psychoanalyst this is a completely different data point different work from words exchanged and thinking in words because words are deterministic tools they're not flows and they are relatively slow they're averaged out and i think that you know just at least thinking opening the conversation about using these additional data points in addition to the narrative from the patient and our feelings and inference about their feelings would possibly allow us you know to work with the phenomenon here that is chaotic dynamic emergent complex because we we base so much on the words and i think that's a very limited uh data data set uh i don't know if that answers

your question albert uh yeah sure definitely but also what about like the the question the patient asks for some questions are really complex and are very difficult to measure using that kind of data like for example a patient comes to the clinic and says um i lack meaning in my life how do you measure that kind of stuff well i i don't mean to trivialize things that i'm going to do an mri if they like meaning in their life certainly we will talk to them just like neurologists talk to them but you know sometimes uh i hear about the lack of meaning when the patient is in a depressed state because when the seeking system is down regulated there is no hope there is no uh you know um enthusiasm but also it's drained of meaning the functional system of salience network you know assigns meaning to things that are have you know uh values for us and and down regulate other a dog in the field will have value for uh a bone and value for a female dog but no value for oak tree right now uh this is a i'm sorry reductionistic example but i do think that in addition to a global philosophical high level discussion what's the meaning of my life there is you know other components of this question of how does a patient talk about meaning and and you could use additional data points about that for example when they are depressed they will tell you that their life is meaningless and you can discuss that this is a phase of their life by the way not a disease of major depression but a mental state so um is that is that answer yeah thank you thank you um

can you hear me okay i really think that the language theory uh that is um that is used here is extremely naive uh for example on page 11 um you say alexai the world apple is meaningless to a baby who never saw or touched an apple this is this is to me not really informed i mean even not going into psychoanalysis of course an apple can be very meaningful even when it is not seen or touched it is when when the when the surroundings when a mother and the father speak about the apple uh it becomes meaningful child and the child will hear it will filter out the world and word and will try to make sense of it with it with the context so um you you could say indeed for example that um this is all coincidence etc but there are several things um of course i had i give you here three examples now there are thousands of these examples this is not just three examples i mean this is empirical stuff that clinicians hear in their very very privileged position of listening to the intimacy of the patient we have a possibility of observing what no one else can observe and this empirical stuff uh um we should do hunger to it we should we should do uh as we are in let's we have like a microscope that nobody else has um freud when he first encountered these um uh ambiguity um elements which are clinically meaningful of course he said like this is nonsense but it is not because it is nonsense that uh and it goes against all the theories that it does not exist like uh charcot used to say it is because darwin was really really uh drawing everything he saw with ultimate precision that and he gathered data and he gathered a lot of data of data that he made up a theory the theory uh had only a chance to exist it's the only chance that it exists because you are drawing reality point by point so your data driven model i couldn't agree more you have this this is the chance where the data the clinical data have a chance of existing and and this is the way science work you make a theory tarwin made a theory that went against all rational all uh evident um um understanding of the world at that point and freud speaks about the the primary process and he says this is something we we find ridicule and we scorn and we laugh about it but still

it is there and so he he made a theory darwin based upon all kind of data which he gathered and then he he had the um the courage to make his theory this theory that he made which was not yet proven allowed to uh experimental researchers the first of one which was mendel to uh devise and think of such an experimental uh uh um uh setup that could uh falsify his theory this experimental setups could not be thought without this theory they needed that theory to make up that experiment so it is a pity that of course generations of clinicians do not like experimental research but if i i can uh share again my screen

uh i do think that there is a big future for experimental psychoanalysis and uh this is ex this is uh precisely what i am doing and um we are finding measures to falsify falsify test uh

uh i apologize is there any chance i could just finish just one more minute this is the geocad and shows we don't have primary processing in acute psychosis and in uh psychic transparency this is a laboratory induced uh setup to induce parapraxis and it shows that parapraxis are defensively motivated and this is another um uh publication about the ability of people solving rebus without knowing it which is one of the elements that we should um um suppose if people have this clinical um uh this clinical uh manifestations so um uh at least at the very least even if you can kind of

relativize and and and draw uh put away all these these these elements as coincidence at the very least as a scientific honesty we should take this hypothesis seriously and test it that is scientifically honest so i did not mean to interrupt you it's just my work in memory buffer was getting full from the

previous episode so when you move to slides i started getting packet drops and i forgot to i'm writing things down but i wanted to because you said something very important before i want to go back to first of all you made a comment about the apple and i'm sorry if it sounds uninformed i happen to be a father and so i've experienced you know uh fatherhood and uh i want to be specific about what i mean before we criticize it is you know babies from birth have reflexes they have rooting reflex they have orienting reflects and other things now when when baby that doesn't have you know visual system developed enough to perceive an apple it is certainly meaningless because she can't like it can detect sweet versus sour and it can orient but it doesn't you know then there is no apple because you can't detect it and then when it sees something apple versus a cup you know if i what's stated in the paper is the word apple okay the word apple is a

bunch of sounds to a baby that doesn't understand the meaning it cannot decode it it may hear mommy and daddy say

that but also mommy and daddy are using million other words and all i'm saying is when we say meaning that means you can

just decode you can see what it means and there's some chain of associations and other than mommy or daddy using the word apple once i don't think you have much but when the baby bites an apple and experiences

you know through personal experience the taste and the crunch and then the the word apple is associated with tactile gustatory you know olfactory experiences then you have another ball game and certainly there is no

bright line but i meant that you need to have long-term memory to make sense of the world that's all it

means just to be specific another thing is i certainly don't say let's throw away all the theories absolutely that's not the point as kurt levin said a good theory is the most practical thing you can have uh that being said i want to be precise about theories and specific so and uh for example uh when people use the

term etiology such as a certain word like black beetle was etiological and panic okay etiology means causal i don't think there's any grounds to say that the word black beetle is causal for a panic attack panic attack is a complex multiply determined phenomenon that is related to bed nucleus austria terminalis anterior cingulate cortex you know perhaps it may have some triggers you know but it's a complex phenomenon that should not be reduced to being triggered by a certain word in panic disorder even if we look at clinical things the definition of a panic attack is that it comes unexpected there is no trigger if it is triggered then technically we're not talking about a panic disorder so i'm just saying that you know in order for us to test something it needs to be falsifiable otherwise we're we cannot test a statement you can certainly test it's like falsifiable i've been tested and i've published it well i'm saying it is as part of your work you know so when you have work with 1400 students and you've conducted the uh the uh french per um uh rebuses the the complex tasks i think what you've proven with your research is that people can can unconsciously solve the rebuses and that is valuable addition to other work which mark solomon cites with uh uh to kidoscope where we can unconsciously read simple things we can count simple things we can associate face with a word unconsciously that's all it means but it doesn't prove that lacan is correct in saying that unconscious structure is a language that is not a falsifiable statement that unconscious is structured as a language that's a belief yeah you can put it in in different parts and and test different parts this is the first part of it is important because it's never supposed it's not into psycholinguistics you only have the hypothesis because you listen to people you dare you need psychoanalysis it's not into psycholinguistics well i think that we test all our medications on rats and mice and you can have a phobia in a mouse you can have an anxiety disorder in the mouse and other things this is what all pharmacology is based on zero language zero so when lacan says unconscious is structured as a language

what unconscious is a vast space i have more memories of how my quad moves when i get up from the chair i have phobic memories i have all sorts of things in my unconscious and i think it's a reduction and it's a certain prism of looking at things to say it's structured as a language and language has multiple structures you know so you know uh english is structured differently than russian you know and so it's culturally determined so i i just think that it's one of the possible avenues of work

i think if we decode and decompose the studies we can you know investigate the important part is also falsifiability as brian nosick showed in 19 2015 36 of studies in psychology are reproducible vast majority of the studies we do when an independent team of researchers repeats it does not show the repeated result so a single study is also a data point but but i'm saying that you know uh theories are good hypotheses are good research is good all of that is good but but the point of my discussion was that narrative fallacy exists it influences clinical work let's be precise about it

i'm not saying let's not use language by all means let's use language but we need other things is what i'm trying to see

wow a lot to it um perhaps um each people david you can unmute and then each person can um make kind of some closing comments from here so david then arian then albert then lights unmute please david thank you

um i think it's uh really helpful that um the other discussants

uh uh prepared questions from the paper um uh i i fully agree about the limitations of all of our models

[illegible]

for that aspect of a given patient,
that those are much more meaningful than a descriptive
case formulation.

So I just wanted to add that.

I liked that part of the paper, including answers for,
and that's all I'll say.

Thank you.

Albert, then Arjan, then I'll make some comments before you, Luxy.

I'm not sure what I can say,
what I've already said before.

I think the discussion proves that the paper opens up a platform for a great discussion.

Maybe, like to add a closing remark,
maybe this is one of the defensive statements,
but to what extent is therapy a science and not an art?
That's a complicated question, but there you go.

Thank you, everybody.

Thank you, everybody.

Yes.

I still had one point in what I had prepared,
and maybe I can bring that point in as a closing remark.
So, Alexei, you also say that the key point of this paper

is that we should not be constrained in designing clinical models
by their need to fit the limitations of our working memory.

And it comes to me that if the clinical work is indeed
very much more centered on helping the patient
to hear what he says in all its ambiguity,
which for me is an important part of the work
next to the transference,
then that changes also that point.

that point because a big part of the clinical work
then is on the primary process level,
and it is what is called free-floating attention.

And that does not weigh upon the working memory
because it's actually letting go of secondary process control
and inhibition, which is very costly.

And so, yeah, I understand that your point is about a fallacy,
but to me, the two points are actually, let's say, obviated.

Whatever the patient says, it is important,
and it makes the patient move.

And by using this angle,
you're not weighed upon by working memory
at the level of your working memory.

And also, but that would be another paper,
I guess, in another session.

I do think that any technical addition to therapy
is actually counterproductive.

And I do think that we need to replace
any medical or technical apparatus for psychotherapy
to something which we dare to claim as clinicians,
which is very much more human sciences,
and which is very much more about the dynamics
of the mental apparatus.

And I think we should dare to claim
that psychotherapy is about these tools
that are not in our model.

Presence, proximity, unconditionality,
patience, availability, and listening,
of course, in an informed and intelligent way.

You took a risk, Alexei, by inviting me.

I'm sorry.

So this is what I have to say.

Thank you.

It's awesome what you've shared
and complimented what he wrote.

I'll just make a few points for footnotes.

This reminds me a lot of scientific pluralism.

There's methodological pluralism.

There's different measurements.

And how do we even juxtapose something

like as vast as the unconscious

with a brain region like Alexei alluded to?

Like, what is that doing in terms of how it plays out
for people already today?

Also, as like a non-therapist,

I saw a lot of parallels between the question

or discussion, does the therapist have an interpretation

with does the organism have a representation?

Because people debate, well, is it a representation

if it's aware or not aware of the representation?

Does it have a representation or is it a representation?

Like David mentioned, just sitting down in the chair

as a posture that then does not use working memory

or explicit memory as just alluded to.

So I think that the interplay of the different complex features

of cognitive systems and how expertise and experience

and all these other factors that you just pointed

to that last little piece like proximity and availability,

those factors that are going to go like off the health record,

but they're part of the actual moment

that your training is about.

So understanding what that is

and not letting it be like overly reduced

or turned into measurements is really

very rich and important discussion.

So again, thank you, Alexi, for bringing it together

and everybody for joining on.

What's the last word, Alexi?

Well, again, I'm grateful to everyone.

I like that it got a little more intense or heated or something.

And I think that we're just getting warmed up.

This is the beginning of the conversation,
maybe not the end of it.

I welcome critique and commentary and further discussions.

And I'm completely fine with disagreements and pluralism,
as Daniel, you mentioned.

Yeah, it's much more nuanced than what I wrote about,
but I was hoping to even talk about it
because sometimes three journals refuse to publish the paper.

You know, one peer reviewer said, you know,
the best part of your paper is where you cited Freud.

And so meaning everything else is completely useless, you know.

And so this is what I'm up against.

And the fact that we're talking about it is music to my ears.

And thank you all very much.

One quick comment I wanted to say about the tools.

I'm not the kind of guy who says,
let's dump the humanity and introduce the tools.
Let's diagnose everything with AI.

That's not the idea.

The idea is to simply let's not throw away information.

If I can get additional information about the patient,
if I know that they've tested their thyroid last month and it's okay,
I'll take it.

You know, I, you know, when experiential people say,
I don't ask questions, I see what happens between me and the patient.

And I draw conclusions based on that.

I think that's a flawed technique because you need to know some things.
I need to know if there was a prior suicide attempt and et cetera, et cetera.
And what medications they're taking.

So I'll take any and all information available to me in whatever form,
visual, auditory, whatever, to help, you know, have a more nuanced picture.

But again, thank you all very much.

I really appreciate it.

Thank you, Daniel.

And, you know, I think.

Thank you.

Till 74.2.

Thank you.

Thank you.

Bye bye.

Bye.

Bye.

Bye.

Bye.